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**U.S. DEPARTMENT OF HEALTH AND HUMAN SERVICES  
CENTERS FOR DISEASE CONTROL AND PREVENTION**

Special Pathogens Branch

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**FIELD TRAINING MANUAL  
ESIAS Case Registration and Support  
A Training Guide for State and Local Public Health Personnel**

Manual No.: CDC-SPB-ESIAS-2022-03

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Issued by: Special Pathogens Branch, CDC (guidance only)

Training: One-day (8-hour) course with written assessment

Status: For Official Use Only

## HOW TO USE THIS MANUAL

This manual trains state and local public health personnel in the registration and support of individuals presenting with Endogenous Silver Ion Anomaly Syndrome (ESIAS). It is a working guide, not a classified report and not a federal directive. Each state and locality administers its own program; this manual describes recommended practice.

## TRAINING OBJECTIVES

By the end of the one-day course, you should be able to:

1. Explain how a case comes to your attention and what you may — and may not — do with that information.
2. Register a case correctly and maintain confidentiality under applicable law.
3. Conduct a first contact that respects the subject's rights.
4. Obtain or respect consent.
5. Provide support appropriate to the subject's category.
6. Know whom to consult when a question of privacy or disclosure arises.

### 1. YOUR ROLE: FEDERAL, STATE, AND LOCAL

CDC provides technical guidance and maintains the confidential registry.

The state health department reports cases to CDC and coordinates services within the state.

Local health departments and community clinics conduct first contact and day-to-day support.

States decide how to staff and fund these activities. This manual describes a recommended practice, not a federal mandate.

**DO NOT:** You are not a federal investigator. You are a public health worker. Your job is support, not enforcement.

### 2. HOW CASES COME TO YOUR ATTENTION

ESIAS cases are identified through the ordinary public health reporting system. There is no separate hidden police file, and no law-enforcement database feeds this program.

Clinical laboratories and hospitals report anomalous findings to the state or local health department, as required by state law.

The state health department reports to CDC through the national notifiable-disease channel.

There is no public reporting channel for this condition. A subject is identified only when the anomaly is found during a health examination or hospital visit for another reason; the subject is not previously aware of the classification.

Privacy note: medical information is protected. In the United States, health information is not routinely shared with law enforcement. Do not seek or accept case information from police files, and do not provide case information to police without a lawful basis.

**DO NOT:** Health data and police data are separate. Do not request case information from law enforcement, and do not release case information to them without a lawful basis.

### 3. REGISTERING A CASE

Open a case file upon confirmation. File number: state code—year—sequence.

Keep the file in a locked cabinet or the confidential registry.

Limit access to staff assigned to the case.

Do not include in the file any information beyond what is needed for the case.

### 4. FIRST CONTACT

First contact is made by two staff members. Its purpose is to offer support, not to investigate.

#### Step-by-step checklist

- ☐ 1. Identify yourselves and state the purpose of the visit.
- ☐ 2. Describe, in plain language, the support options available.
- ☐ 3. Explain the known medical facts (Section 5).
- ☐ 4. Ask whether the subject wishes to receive services.
- ☐ 5. Respect a decline. Offer a voluntary non-participation form.
- ☐ 6. If undecided, you may schedule a second visit within two weeks.

If the subject or family use folk or informal language (for example, calling the condition a "curse"), do not repeat it. Explain in neutral medical terms. Do not offer a mystical explanation.

**DO NOT:** Do not use force. Do not require the subject to accept help. Do not continue to contact someone who has clearly declined.

### 5. CONSENT

Explain, in plain words: silver in the blood at a non-pathological concentration; the link between mood and health; and the free services available. The subject signs a consent form if they choose to receive services. They may decline or withdraw at any time.

### 6. CONFIDENTIALITY

Case information is confidential under applicable state and federal law. Unauthorized disclosure may have legal consequences. Confidentiality is a continuing obligation of your employment; it is not a separate "lifetime pledge." Do not discuss cases in public, do not disclose information to family or a spouse without the subject's consent, and do not retain case material outside authorized systems.

### 7. SUPPORT BY CATEGORY

#### Category 1

Offer routine follow-up, physiological monitoring, mood-management guidance, and help with housing and employment, subject to consent.

#### Category 2

Help the subject establish a stable social role — for example, quiet, low-interaction employment. Registration and a legal address may be arranged.

#### Category 3

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Handle by non-contact only. Do not approach, tag, or interview. A designated program places basic supplies at a set point each year; staff confirm the subject is present only by whether the supplies are taken. Territory coordinates are held by the state program, not by local field staff.

### Category 4 and 5

No confirmed living individual in the continental United States. If such a case is reported, notify the state health department immediately.

## 8. SPECIAL SITUATIONS (PRIVACY AND DISCLOSURE)

Case information is confidential. Do not disclose to anyone — including a spouse, partner, or family member — without the subject's written consent. This manual does not prescribe a standard disclosure ceremony. Where a disclosure question arises (for example, in connection with marriage or employment), consult the state health department or the agency's legal counsel; state law governs privacy and the release of information. A release-of-information form (Form 5) may be used where the subject consents.

## 9. TRAINING AND ASSESSMENT

Complete the one-day (8-hour) course, including this manual, communication, confidentiality, and case examples. Pass the written assessment at the end of the course. States may adjust the format. Refresher training is recommended every two years.

### Assessment (sample)

1. When a case is identified, who reports it to CDC? ( )  
A. The police B. The state health department C. The subject D. The local clinic directly
2. May you release case information to a spouse without consent? ( )  
A. Yes B. No — consent is required C. Only in marriage D. Only to family
3. How is a Category 3 subject monitored? ( )  
A. Approached monthly B. By satellite C. Non-contact; supplies taken D. By police patrol
4. A family member calls the condition a "curse." You should: ( )  
A. Agree B. Use neutral medical terms C. Call a clergyperson D. Leave
5. Where does the law on privacy and release of information come from? ( )  
A. This manual B. State law and legal counsel C. CDC alone D. Police policy

## 10. FREQUENTLY ASKED QUESTIONS

### Q: What if a subject refuses services?

A: Respect the decision. Offer the voluntary non-participation form. Do not contact the subject again unless there is a clear public-safety risk, in which case the matter is reviewed by a committee.

### Q: What if a family member calls the condition a "curse"?

A: Do not repeat the term. Explain in neutral medical terms. Do not mystify.

### Q: May we tell a spouse or partner?

A: Only with the subject's written consent. Consult legal counsel on state-specific requirements.

### Q: Do subjects lose the right to travel?

A: No. Subjects retain full civil rights, including freedom of movement.

## FORMS

### Form 1 — Case Report

Case No.: \_\_\_\_\_ Date: \_\_\_\_/\_\_\_\_/\_\_\_\_\_  
Reporting agency: \_\_\_\_\_ State: \_\_\_\_\_  
Source: ☐ Lab ☐ Hospital ☐ State screening ☐ Self ☐ Referral  
Category: ☐ 1 ☐ 2 ☐ 3 ☐ 4 ☐ 5  
Reported by: \_\_\_\_\_ Reviewed by: \_\_\_\_\_

### Form 2 — First Contact Record

Case No.: \_\_\_\_\_ Date: \_\_\_\_/\_\_\_\_/\_\_\_\_\_  
Contact: ☐ Field visit ☐ Office ☐ Other  
Consent explanation: ☐ Given ☐ Deferred  
Subject response: ☐ Accepted ☐ Declined ☐ Unsure  
Staff: \_\_\_\_\_ / \_\_\_\_\_

### Form 3 — Consent / Non-Participation Form

I have received an explanation of the known medical facts and the services available to me. I consent to / decline (circle) follow-up services.  
Signature: \_\_\_\_\_ Date: \_\_\_\_/\_\_\_\_/\_\_\_\_\_  
Witness: \_\_\_\_\_

### Form 4 — Confidentiality Agreement

I agree not to disclose case information during or after my employment.  
Name: \_\_\_\_\_ Agency: \_\_\_\_\_ Date: \_\_\_\_/\_\_\_\_/\_\_\_\_\_  
Witness: \_\_\_\_\_

### Form 5 — Consent for Release of Information

Case No.: \_\_\_\_\_ Date: \_\_\_\_/\_\_\_\_/\_\_\_\_\_  
I, the subject, authorize release of the following information to [name]: known medical facts and available services.  
Signature: \_\_\_\_\_ Date: \_\_\_\_/\_\_\_\_/\_\_\_\_\_  
Witness: \_\_\_\_\_

### Form 6 — Training Sign-In

Course: ESIAS Field Training (One-Day) Date: \_\_\_\_/\_\_\_\_/\_\_\_\_\_  
Name      Agency      Signature  
1. \_\_\_\_\_  
2. \_\_\_\_\_  
3. \_\_\_\_\_  
4. \_\_\_\_\_  
(Continue on additional sheet)

— End of Manual —

Issued by the Special Pathogens Branch, Centers for Disease Control and Prevention

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